Northeast Hematology Institute
Dept. of Hematology
40 University Research Drive, Burlington, VT 05401

Discharge Summary

Patient:	Lothar Herrmann
Date of Birth:	10/31/1948
Admission Date:	09/01/2025
Discharge Date:	09/10/2025
Attending Physician:	Prof. Lorenz Trümper, MD

Dear Colleagues,

Mr. Lothar Herrmann, DOB 10/31/1948, was admitted on 09/01/2025 to initiate ABVD chemotherapy for newly diagnosed classical Hodgkin lymphoma.

Mr. Herrmann presented 4 weeks ago with a 3-month history of progressive bilateral cervical and supraclavicular lymphadenopathy, drenching night sweats, and 6 kg weight loss. Excision biopsy confirmed mixed cellularity classical Hodgkin lymphoma, EBV-positive, CD30+ RS cells. PET-CT: bulky mediastinal and bilateral cervical disease. No infradiaphragmatic involvement. Ann Arbor stage IIA (bulky).

Primary Diagnosis: Hodgkin lymphoma (C81.9) — Tumor staging: T2 N2 M0

Laboratory values on admission:

Parameter	Value	Reference
HbA1c	5.5%	< 7.0%
LDH	380 U/L	< 250 U/L
Hemoglobin	11.8 g/dL	13.5–17.5 g/dL

Past Medical History: Well-controlled type 2 diabetes on gliclazide 60 mg and metformin 1000 mg twice daily (HbA1c 5.5%). No cardiac disease. Right inguinal hernia repair 2016. Retired accountant, non-smoker, 2–3 units alcohol per week.

Cardiac Assessment Prior to Anthracycline: Echo: EF 64%, no significant pathology, no amyloid features. ECG: normal sinus rhythm. No contraindication to doxorubicin identified. Pulmonary function: FEV1 78% predicted, FVC 82% predicted — acceptable for bleomycin initiation with close monitoring.

Hospital Course: Cycle 1 of ABVD (doxorubicin, bleomycin, vinblastine, dacarbazine) administered over 2 days with standard antiemetics (ondansetron 8 mg IV, dexamethasone 8 mg IV). Grade 1 nausea day 2, managed with oral ondansetron. No vomiting. Appetite returned day 3. No febrile episodes.

Glycaemic Management on Dexamethasone Days: Blood glucose monitoring intensified. Gliclazide dose temporarily increased to 80 mg once daily on steroid days. Fasting glucose remained below 200 mg/dL throughout. No insulin required.

Allied Health and Patient Education: Oncology physiotherapist provided advice on maintaining physical activity during chemotherapy and managing fatigue. A graded exercise plan was given. Dietitian confirmed satisfactory nutritional status (BMI 24.8) and provided dietary advice for managing chemotherapy-related nausea. Family meeting day 5 with Mr. Herrmann and his wife addressed prognosis and treatment side effects — both reported reassurance regarding the cure rates for early-stage Hodgkin lymphoma. Both were given the hematology specialist nurse's direct contact details for questions between appointments.

Medication at Discharge: Gliclazide 80 mg once daily (maintained increased dose), metformin 1000 mg twice daily, ondansetron 8 mg as needed, omeprazole 40 mg.

Follow-up: Hematology in 2 weeks for pre-cycle 2 assessment. Interim PET-CT after cycle 2. Pulmonary function testing to be repeated after 4 cycles. Patient enrolled in the Hodgkin lymphoma clinical pathway with 24-hour emergency contact number.

Pre-Treatment Cardiac and Pulmonary Assessment — Detailed Results:

Transthoracic echocardiography confirmed EF 64% with no significant valvular pathology, normal LV dimensions, no amyloidosis features on strain analysis, preserved right ventricular function, and no pericardial effusion. ECG: normal sinus rhythm at 68 bpm. Pulmonary function testing showed FEV1 78% of predicted, FVC 82% predicted, FEV1/FVC 0.74, and DLCO 68% of predicted — slightly below the lower limit of normal. The haematology team discussed with the patient the importance of monitoring for pulmonary bleomycin toxicity, including symptoms to report (new or worsening dyspnoea, dry cough, exercise intolerance). PFTs will be repeated after cycle 4 and bleomycin discontinued if DLCO declines by more than 20% from baseline.

Glycaemic Management Plan for Subsequent ABVD Cycles:

A structured glycaemic management plan was developed for future cycles given the dexamethasone pre-medication on chemotherapy days. The plan specifies: blood glucose monitoring before meals and at bedtime on dexamethasone days; gliclazide to be increased from 60 mg to 80 mg on dexamethasone days and returned to 60 mg on non-steroid days; a glucose threshold of 250 mg/dL above which the patient should contact the diabetes team for sliding scale insulin advice; and metformin to be held on days when IV contrast CT is planned and for 48 hours afterwards. The plan was documented and provided to the patient in written form with examples.

Patient and Family Education:

An extended family meeting was held on day 5 with Mr. Herrmann and his wife. The haematology specialist nurse and the treating physician attended. Topics addressed: prognosis for early-stage classical Hodgkin lymphoma and the expected outcome with ABVD; the planned 6-cycle treatment schedule; the interim PET-CT response assessment after cycle 2 and its significance; side effects to anticipate including nausea, fatigue, hair loss, infection risk, and bleomycin lung toxicity monitoring; and practical guidance on managing side effects at home. Both Mr. Herrmann and his wife reported feeling reassured regarding the prognosis and expressed confidence in the treatment team.

Prof. Lorenz Trümper, MD

Director, Dept. of Hematology